3.14 Mechanical Ventilation*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective *No specific complaint* **Objective** • Endotracheal tube in place • Secretions in the tube • High-pressure alarm • Coarse crackles	**ACTUAL** Ineffective airway clearance related to the presence of an artificial airway and inability to cough as evidenced by secretions and high airway pressures.	• Airway patent and breath sounds clear.	• Auscultate every 2–4 hours. • Suction when indicated, with sterile technique and pre-oxygenation. • Check the tube marking at the lips and the cuff pressure every shift. • Answer ventilator alarms at once. • Humidify inspired gas.	• Goal reached when breath sounds are clear and pressures normal.
Subjective • Nods but cannot speak **Objective** • Intubated • Gestures in frustration	**ACTUAL** Impaired verbal communication related to the endotracheal tube as evidenced by inability to speak.	• Makes needs known by other means.	• Ask yes/no questions. • Provide a writing board or picture chart. • Explain every procedure before starting.	• Met when needs are communicated.
Subjective *No specific complaint* **Objective** • Intubated more than 48 hours • Lying supine • Sedated	**RISK** Risk for infection as evidenced by risk factors: invasive airway and impaired cough (ventilator-associated pneumonia).	• No ventilator-associated pneumonia.	• Keep the head of the bed at 30–45 degrees. • Give antiseptic oral care per protocol. • Review each day whether sedation can be paused and weaning begun. • Drain condensate away from the patient. • Use closed suction, per policy.	• Achieved if there is no new fever or infiltrate.

* **References for this care plan:** [35], [37], [44], [128]. Full entries are listed in the References section of the CPMN book.